

A FREEBIE FROM FEMME ON THE SPECTRUM

# The 12 Signs of Late-Diagnosed AuDHD in Women

(the ones your therapist probably missed)

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By Liz Bucher — a late-diagnosed autistic, ADHD mom of two, current MSW student, and the host of the Femme On the Spectrum podcast.

## A quick honest note before we start:

I'm not a licensed therapist (yet — working on it). This is not a diagnostic tool. Nothing in this guide will tell you whether you are autistic, ADHD, AuDHD, or something else entirely. Only a clinician trained in adult and women's neurodivergence can do that.

What this guide IS: a clear-eyed, lived-experience map of 12 patterns that show up again and again in late-diagnosed AuDHD women. If you read them and feel seen, that feeling is allowed to matter. It might be the start of something. Trust yourself.

P.S. — If you took the quiz at [femmeonthespectrum.com](https://femmeonthespectrum.com) that led you here, your personalized results are in your inbox too. Read this guide first, then check.

# How to read this

Each sign has three parts: the description (what the sign actually is), *what it looks like on the outside* (so you can recognize it), and *what's actually happening* (so you can stop blaming yourself for it).

You don't need to relate to all 12 to be AuDHD. Most late-diagnosed women I know relate strongly to 6-10. Some hit you in the chest immediately; others creep up later. Read at your own pace. Highlight what feels true. Skip what doesn't.

And here's the most important rule of this whole document: **this is not a diagnostic checklist. It's a doorway. Walk through it at the pace that works for you.**

## #1

### You're a chameleon — and you're exhausted

You've spent your whole life studying people. Watching what they laugh at, copying their gestures, building scripts for every social situation. You've gotten very good at it. Too good — most people have no idea you're doing it. You don't always know you're doing it.

#### WHAT IT LOOKS LIKE

*You come home from a perfectly nice gathering and want to lie face-down on the floor for two hours. Your face hurts. You can't remember if you actually had a good time or just performed one.*

#### WHAT'S ACTUALLY HAPPENING

Welcome to masking. It's a survival skill, and you're a master of it. It's also exhausting in a way that most people will never understand — because most people aren't running a full simulation of themselves in real time.

## #2

### Your senses are loud (and have always been loud)

Tags on the inside of shirts. Fluorescent lights. Someone chewing across the room. The hum of a fridge. Anyone wearing too much perfume. You're not making it up. Your nervous system is genuinely processing more sensory information than the average person's.

#### WHAT IT LOOKS LIKE

*You've been called "picky" since childhood. You have strong opinions about textures. You can't focus when there's background noise. A bad smell can ruin your entire day.*

#### WHAT'S ACTUALLY HAPPENING

You weren't being dramatic. You were being autistic. Sensory sensitivity is one of the most underrecognized markers of autism in women — partly because we get told from age four that we're "too sensitive" and learn to mask it.



## #3

## Socializing has a hangover

Other people get a little tired after a party. You need 36 hours, a weighted blanket, and a moratorium on all human contact. This isn't introversion — introverts recharge in 2-3 hours. This is the cost of running your masking simulation at full power for that long.

**WHAT IT LOOKS LIKE**

*You cancel plans the day-of more than you'd like to admit. You've ghosted group chats. You've cried in the car after a baby shower. You schedule "recovery days" after social events.*

**WHAT'S ACTUALLY HAPPENING**

It's not that you don't love people. It's that the version of you that shows up to events isn't unmasked-you, and that performance is metabolically expensive. You're not flaky. You're spent.

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## #4

## Your special interests are not a phase

When you find a thing, you find it. You don't dabble — you research, document, organize, evangelize. You have detailed opinions about a topic the average person doesn't know exists. The intensity of your interest is the entire point — it's where joy lives.

**WHAT IT LOOKS LIKE**

*Your camera roll is 60% screenshots of one niche thing. Your closest friends have learned to brace themselves when you say "so I've been reading about—". You've considered going back to school for it.*

**WHAT'S ACTUALLY HAPPENING**

Special interests are autistic joy in its purest form. They are not childish, they are not frivolous, they are not something to grow out of. They are how your brain is built to engage with the world.

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## #5

## You've been called "too much" your whole life

Too intense. Too sensitive. Too quiet. Too loud. Too smart. Too weird. Too direct. Too soft. Whatever you are, someone has told you it's too much of it. So you spent the next 20 years calibrating yourself into something palatable — and you're tired.

**WHAT IT LOOKS LIKE**

*You over-apologize for taking up space. You preemptively shrink. You watch yourself in conversations like there's a director critiquing your performance.*

**WHAT'S ACTUALLY HAPPENING**

You were never too much. You were unmodulated for a world that has very narrow tolerances for women who don't perform femininity in the expected key. You're allowed to take up your full size.

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## #6

## Plans changing wrecks you

Someone moves a meeting by an hour. Your kid's daycare schedule shifts. The restaurant is closed when you get there. To everyone else, it's a minor adjustment. To you, it's a controlled demolition of the entire afternoon.

**WHAT IT LOOKS LIKE**

*You build extensive systems and rituals around your day. You hate spontaneous plans. You need to know what's happening, when, and what comes after. You'd rather skip the thing than go to a different version of the thing.*

**WHAT'S ACTUALLY HAPPENING**

Routine isn't a personality quirk; for you it's a regulatory tool. The structure is what keeps your nervous system in a livable range. You're not rigid — you're protecting yourself from sensory and executive overload.

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## #7

## You can't always tell what you're feeling

Someone asks how you're doing and you genuinely don't know. There's a feeling somewhere — chest tight, stomach off, jaw clenched — but the word for it is locked in a different drawer. By the time you figure it out, it's three days later and you're sobbing about something tangentially related.

**WHAT IT LOOKS LIKE**

*Therapists describe you as "intellectual" or "in your head." You journal a lot, trying to translate body signals into language. You sometimes feel emotionally numb, then suddenly flooded.*

**WHAT'S ACTUALLY HAPPENING**

This is called alexithymia, and it's not the same as not having feelings. Your feelings exist; the translation layer between body and word is just slower or more inconsistent. It's extremely common in autistic women and is highly underdiagnosed.

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## #8

## You rehearse conversations (before, during, and after)

Before: you script the meeting, the call, the small talk. During: you're half in the conversation and half in your head, running the script. After: you replay every single moment, agonizing over what you said, didn't say, the tone, the eye contact.

**WHAT IT LOOKS LIKE**

*You can't fall asleep because you're auditing a five-minute interaction from 11 a.m. You write follow-up texts and don't send them. You have a folder of unsent apologies.*

**WHAT'S ACTUALLY HAPPENING**

Your social processing is not a flaw — it's a backup system that's been compensating your whole life for a wiring that doesn't read social signals automatically. The exhaustion is real. The good news is, when you stop masking, the backup system gets to rest.

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## #9

## Burnout, but not the kind they mean

Autistic burnout is not regular burnout. It's a chronic exhaustion that comes from years (decades) of masking, sensory overload, and operating in a world that wasn't built for your nervous system. It can take you out for months. It feels like depression but doesn't respond to depression treatments.

**WHAT IT LOOKS LIKE**

*You used to be high-functioning and now basic tasks feel impossible. Showering is a project. You forget words. You have "shutdowns" where you can't speak or process information. Doctors keep saying it's anxiety. It's not.*

**WHAT'S ACTUALLY HAPPENING**

This is one of the most misunderstood experiences in adult autism. Rest, sensory accommodation, and permission to unmask are the cure — not more productivity hacks. If this sounds like you, please be gentle with yourself.

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## #10

## You have a deep, almost physical sense of justice

Unfairness lights something on fire in you that other people seem able to shrug off. You can't NOT speak up. You'll lose sleep over a stranger being treated badly on the bus. You've quit jobs over ethical issues that your coworkers "learned to live with."

**WHAT IT LOOKS LIKE**

*You're "the office troublemaker" because you keep pointing out things that aren't right. You can't stay in friendships with people who treat staff badly. Politics is not optional for you — it's structural.*

**WHAT'S ACTUALLY HAPPENING**

Strong moral/ethical sensitivity is a recognized autistic trait. It is not a personality flaw. It's also why you'd make a phenomenal advocate, organizer, or therapist — fields that desperately need people who refuse to look away.

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## #11

## You've collected diagnoses that didn't quite fit

Anxiety, depression, bipolar II, BPD, ADHD (maybe a real piece, but not the full picture). The medications helped a little, until they didn't. Therapy helped some, but kept circling around the actual thing. You've started to suspect they were treating symptoms while missing the system underneath.

**WHAT IT LOOKS LIKE**

*You've been in and out of treatment since adolescence. You suspect your file is thick. You've tried more than three SSRIs. You've heard "have you considered that you might just be highly sensitive?" and wanted to scream.*

**WHAT'S ACTUALLY HAPPENING**

Autistic women are wildly underdiagnosed and frequently misdiagnosed. If your treatment history reads like a wild goose chase, it might be because the geese (geese?) were all decoys. The thing under the symptoms could be something the system isn't trained to recognize in women.

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## #12

## Finding the words felt like coming home

The first time you read a thread by a late-diagnosed autistic woman, you felt something crack open. You weren't broken. You weren't "too much." You weren't a failed normal person. You were a successful version of a different thing entirely — and there was a name for it, and there were others.

**WHAT IT LOOKS LIKE**

*You went down a 4 a.m. rabbit hole and have read everything since. You cried watching a TikTok. You finally said it out loud to someone, voice shaking. You signed up for a quiz. You opened this PDF.*

**WHAT'S ACTUALLY HAPPENING**

If you're here, something already brought you here. Trust that. You don't need a formal diagnosis to begin honoring how your brain actually works — though if a formal diagnosis would help you access support, accommodations, or simply your own self-trust, please pursue one. Either way, welcome. We've been waiting for you.

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## If this resonated...

Here's what I'd do if I were you, in roughly this order. Don't try to do them all at once. Pick one. Come back to the rest when you're ready.

### 1. Sit with it.

Take a day. A week. Whatever you need. You just looked at something hard. You don't need to decide anything right now.

### 2. Listen to the podcast.

I made *Femme On the Spectrum* for women who scored exactly like you. Real conversations, no fluff, no clinical jargon. Start anywhere — there's no required reading list.

### 3. Talk to a friend or partner.

Saying "I think I might be autistic" out loud to one safe person is one of the hardest and most freeing parts of this. Choose someone you trust. You'll know who.

### 4. Read more.

A few books I recommend: *Unmasking Autism* by Devon Price, *Divergent Mind* by Jenara Nerenberg, *Sincerely Your Autistic Child* by the Autistic Women & Nonbinary Network. (More on the podcast website.)

### 5. Consider a formal evaluation.

If formal diagnosis would help you access support, accommodations, or your own self-trust, it's worth pursuing. Look for clinicians who specialize in adult women or AuDHD — not all clinicians are trained to recognize how autism presents in us. (Lots of practitioners list "adult ASD assessment, women" specifically.)

### 6. Join my waitlist for when I'm licensed.

I'm currently in my Master of Social Work program and expect to be a licensed therapist taking telehealth clients in 2028. If you want to work with me when that happens, join the waitlist at [femmeonthespectrum.com/waitlist](https://femmeonthespectrum.com/waitlist). Early-access spots will go to the list first.

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## One more thing

I made this guide because nobody handed me one. I went through years of misdiagnoses, wrong medications, three different therapists telling me I had "just" anxiety or "just" depression, before I found my way to my actual diagnosis at 32. I don't want any of that for you.

You are not broken. You are not too much. You are not failing at being a normal person. You are very possibly succeeding at being a different, beautiful, complicated thing that the world is finally — finally — learning to see.

I'm glad you're here.

— Liz

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Femme On the Spectrum • Hosted by Liz Bucher (MSW in progress, expected licensure 2028)

Podcast: [femmeonthespectrum.com/episodes](https://femmeonthespectrum.com/episodes) • Waitlist: [femmeonthespectrum.com/waitlist](https://femmeonthespectrum.com/waitlist)

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